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The Honorable Thomas O. Rice

IN THE UNITED STATES DISTRICT COURT
FOR THE EASTERN DISTRICT OF WASHINGTON
AT SPOKANE

JANE ELIZABETH ROBERTS, *et al.*,

Plaintiff,

v.

SHRINERS HOSPITALS FOR CHILDREN, SHRINERS HOSPITALS FOR CHILDREN – SPOKANE, BEVERLY BOKOVITZ, FRANCES FARLEY, JERRY GANTT, JOHN MCCABE, PHILLIP GRADY, PETER BREWER, AND GOV. JAY INSLEE,

Defendants.

No. 2:23-cv-00295-TOR

SHRINERS CHILDREN'S MOTION TO DISMISS

2/5/2024
Without Oral Argument

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I. INTRODUCTION

Facing an unprecedented global health crisis, and after Governor Jay Inslee issued a proclamation requiring hospital employees and healthcare workers receive a COVID-19 vaccine, Defendant Shriners Hospitals for Children ("Shriners Children's")[1] instituted a COVID vaccination requirement for employees. It did so in August and September 2021, first at its Spokane facility and then system-wide across the United States and Canada, to protect its pediatric patients and their families, as well as hospital staff and others, from contracting COVID, and to comply with Washington law. By the time Shriners Children's required vaccinations, the U.S. Food and Drug Administration had given full approval to Pfizer-BioNTech's COVID vaccine for use by individuals 16 or older. But the risk to Shriners Children's patients—specifically young children—was acute because there was no approved vaccine for children under 12 and COVID cases among children were surging nationally. Accordingly, Shriners Children's acted lawfully and in the best interests of their pediatric patients and their patients' families by requiring vaccination of hospital staff and others. Plaintiffs' claims to the contrary are wholly without merit.

Plaintiffs are 12 former Shriners Children's employees discharged after they chose not to receive a COVID vaccine. They challenge Shriners Children's

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[1] The two Shriners Hospitals for Children corporate defendants and six Shriners Hospitals for Children officials named as defendants in this lawsuit are referred to collectively as "Shriners Children's" or "Shriners Children's Defendants."

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ability to institute a COVID vaccination policy for employees. Their Complaint spans 104 pages and 417 paragraphs of sprawling allegations, yet their claims are fundamentally deficient and should be dismissed for two primary reasons.

***First*,** Shriners Children's is not a state actor subject to 42 U.S.C. § 1983 ("Section 1983") claims for the alleged violation of constitutional or federal rights. It is a private nonprofit hospital. Adding Governor Inslee as a co-defendant does not transform the Shriners Children's Defendants into state actors, and Plaintiffs' six Section 1983 claims cannot overcome this threshold issue.

***Second*,** independent of the state actor issue, Plaintiffs' 10 total claims all fail as a matter of law for the following reasons:

1. Plaintiffs do not claim they received a COVID vaccine, and a fully authorized vaccine was available at the time of Shriners Children's requirement. Therefore, Plaintiffs cannot bring statutory, regulatory, and treaty-based claims based on theories that they were forced to participate in medical experiments. Moreover, courts have consistently held that there is no private right of action to assert Section 1983 claims under the EUA Act (21 U.S.C. § 360bbb-3). And the PREP Act (42 U.S.C. § 247d-6d), which provides broad immunity from claims related to the administration of covered countermeasures (such as the COVID vaccine) in a public health emergency, does not provide any relief absent serious physical injury from the administration of the vaccine, which Plaintiffs did not receive at all (Counts 1, 4, 6, and 10).

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2. Vaccination requirements are lawful, do not implicate fundamental rights, and need only satisfy the deferential rational basis review standard. *See, e.g., Williams v. Brown*, 567 F. Supp. 3d 1213, 1226-28 (D. Or. 2021); *Bacon v. Woodward*, 2021 WL 5183059, at *4-5 (E.D. Wash. Nov. 8, 2021). Plaintiffs' claims that vaccine requirements violate constitutional rights, including equal protection, due process, and unconstitutional conditions claims, have been routinely rejected by courts considering COVID vaccine policy challenges. Shriners Children's employee vaccine requirement complied with state mandates and was rationally related to protecting patients and their families, staff, and others from spreading COVID (Counts 2, 3, and 5).

3. Plaintiffs' supplemental Washington law claims for breach of contract, unspecified "Washington State Common Law Employment Torts," and outrage fail as well, in particular because Plaintiffs are not parties to or beneficiaries of Shriners Children's Provider Agreement with the Centers for Disease Control concerning administration of COVID vaccines and the institution of a lawful employee vaccine policy cannot constitute "extreme and outrageous conduct" that "go[es] beyond all possible bounds of decency" necessary to establish an outrage claim. *Kloepfel v. Bokor*, 149 Wn.2d 192, 196-97 (2003) (Counts 7, 8, and 9).

Plaintiffs' claims are misguided and contrary to law. As the U.S. Supreme Court explained when rejecting a challenge to a federal COVID vaccination requirement for healthcare workers, "ensuring that providers take steps to avoid

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transmitting a dangerous virus to their patients is consistent with the fundamental principle of the medical profession" to "first, do no harm." *Biden v. Missouri*, 595 U.S. 87, 93 (2022). "It would be the 'very opposite of efficient and effective administration for a facility that is supposed to make people well to make them sick with COVID-19.'" *Id.* (quoting *Florida v. Dep't of Health & Human Servs.*, 19 F.4th 1271, 1288 (11th Cir. 2021)). Shriners Children's employee vaccination policy furthered this lawful purpose. The Court should dismiss Plaintiffs' Complaint with prejudice and without leave to amend what are futile claims.

II. BACKGROUND

Shriners Children's Spokane. Shriners Hospitals for Children, a private nonprofit corporation, operates Shriners Children's Spokane, a 30-bed hospital that treats thousands of pediatric patients annually regardless of their families' ability to pay. *See* Shriners Children's Spokane, Community Needs Assessment, 2022, available at https://doh.wa.gov/sites/default/files/2022-11/CHNA-042-2022_0.pdf?uid=65166699d9821.[2] Shriners Children's Spokane provides care

[2] While external to the Complaint, this document and others referenced in this Motion are properly subject to consideration on a motion to dismiss. *See Metzler Investment GMBH v. Corinthian Colleges, Inc.*, 540 F.3d 1049, 1061 (9th Cir. 2008) (court's review on motion to dismiss is limited to "the complaint, materials incorporated into the complaint by reference, and matters of which the court may take judicial notice"). Judicial notice is proper where the evidence "is not subject to reasonable dispute[.]" Fed. R. Evid. 201(b). Under this doctrine, a court on a

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for children with orthopedic conditions, burns, spinal cord injuries, and cleft lip and palate conditions. *Id.* Shriners Children's Spokane treated a record 16,000 children in its outpatient clinic and performed nearly 1,000 surgeries in 2021. *Id.*

Plaintiffs and Individual Defendants. Plaintiffs are 12 former employees of Shriners Children's Spokane who were discharged in accordance with Shriners Children's policy after they chose not to receive a COVID vaccine. Compl. (ECF No. 1) ¶¶ 18.1-18.12, 171, 400. The individual Shriners Children's Defendants in this action are Beverly Bokovitz (chief nursing officer), Dr. Frances Farley (chief medical officer), Jerry Gantt (president and chairman of board of trustees), John McCabe (executive vice president and chief operating officer), Phillip Grady (vice president of hospital operations), and Peter Brewer (administrator of

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motion to dismiss may consider, among other evidence, "records and reports of administrative bodies," *Mack v. S. Bay Beer Distribs., Inc.*, 798 F.2d 1279, 1282 (9th Cir. 1986), *abrogated on other grounds by Astoria Fed. S&L Ass'n v. Soimino*, 501 U.S. 104 (1991), the content of state and federal regulations, *S. Bay United Pentecostal Church v. Newsom*, 985 F.3d 1128, 1132 (9th Cir. 2021), *vacated on other grounds*, 141 S. Ct. 2563 (2021), and public records and government documents available from reliable sources on the Internet, *U.S. ex rel. Modglin v. DJO Glob. Inc.*, 114 F. Supp. 3d 993, 1008 (C.D. Cal. 2015), *aff'd sub nom. United States v. DJO Glob., Inc.*, 678 F. App'x 594 (9th Cir. 2017); *Daniels-Hall v. Nat'l Educ. Ass'n*, 629 F.3d 992, 998-99 (9th Cir. 2010) (taking judicial notice of information on the websites of two school districts).

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Shriners Children's Spokane). *Id.* ¶¶ 19.3-19.8. Plaintiffs allege collective action by Bokovitz, Farley, Gantt, McCabe, Grady, and Brewer throughout their Complaint, referring to Shriners Children's and the individual defendants as "Shriners PolicyMakers." *Id.* at p.2; *see also id.* ¶¶ 7-8, 273-311. Washington Governor Jay Inslee is also an individual defendant, separately represented. ECF Nos. 23-28.

EUA Authorization for COVID Vaccines. On December 11, 2020, the U.S. Food and Drug Administration (FDA) issued the first emergency use authorization (EUA) for a COVID vaccine, authorizing Pfizer-BioNTech's vaccine to be distributed in the U.S. for use by individuals 16 and older. https://www.fda.gov/news-events/press-announcements/fda-takes-key-action-fight-against-covid-19-issuing-emergency-use-authorization-first-covid-19. The FDA authorization cited safety data supporting the EUA in the form of an international study of over 37,000 participants who received either the vaccine or a placebo, as well as data reporting that the vaccine was 95 percent effective in preventing COVID among trial participants. *Id.*

The FDA issued a second EUA for Moderna's COVID vaccine on December 18, 2020. https://www.fda.gov/news-events/press-announcements/fda-takes-additional-action-fight-against-covid-19-issuing-emergency-use-authorization-second-covid. And the FDA issued a third EUA for Janssen's single-dose COVID vaccine on February 27, 2021. https://www.fda.gov/news-events/press-announcements/fda-issues-emergency-use-authorization-third-covid-

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19-vaccine. As Governor Inslee subsequently reported, as of August 2021, more than 346 million doses of COVID vaccines had been administered in the U.S. (including 8.2 million doses in Washington) and "serious safety problems and long-term side effects are rare." https://governor.wa.gov/sites/default/files/proclamations/ 21-14%20-%20 COVID-19%20Vax%20Washington%20 %28tmp%29.pdf.

Governor Inslee's Vaccination Requirement. On August 9, 2021, Governor Inslee issued Proclamation 21-14 in response to the emergence of "highly contagious COVID-19 variants, including the 'delta variant' that is at least twice as transmissible as the virus that emerged in late 2019," with these variants having "caused COVID-19 cases and hospitalizations to rise sharply among unvaccinated people." *Id.* Proclamation 21-14 required all "Health Care Providers" in the state to be fully vaccinated against COVID by October 18, 2021. *Id.* A "Health Care Provider" was defined to include "[w]orkers in any Health Care Setting," with "Health Care Settings" expressly including "[a]cute care facilities, including … hospitals." *Id.* Governor Inslee reiterated the support for a vaccination requirement for healthcare workers among national medical groups (including the American Medical Association) "noting that such a requirement is the 'fulfillment of the ethical commitment of all health care workers to put patients as well as residents of long-term care facilities first and take all steps necessary to ensure their health and well-being.'" *Id.* The vaccination requirement was intended to protect "vulnerable populations,"

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including "persons in health care facilities" and to "protect our health … care systems themselves." *Id.*

Shriners Children's Notified Staff of Governor Inslee's Requirement. Following Governor Inslee's Proclamation, Shriners Children's notified Spokane staff of the state vaccination requirement. In an all-staff email on August 11, 2021, Shriners Children's Kristin Monasmith wrote: "Coming from the Governor and the state of Washington, the Spokane Hospital is required to abide by the state mandate and follow applicable laws. Shriners Hospitals for Children does not have the ability to adapt or vary from the statements within the proclamation." Compl. ¶ 293. She added that "[e]mployees who refuse to be vaccinated will be subject to dismissal from employment for failing to meet legal job qualifications." Compl. Ex. F (ECF No. 1-6) at 3.

The FDA Provides Full Approval to Pfizer-BioNTech's Vaccine. Two weeks after Governor Inslee's Proclamation, the FDA on August 23, 2021, provided full approval to Pfizer-BioNTech's COVID vaccine for use by individuals 16 and older. https://www.fda.gov/news-events/press-announcements/fda-approves-first-covid-19-vaccine. Pfizer's vaccine was to be marketed as Comirnaty going forward. It remained authorized for EUA use for individuals 12 to 15[3] and as a third dose for certain immunocompromised

[3] The FDA approved EUA use of the Pfizer-BioNTech COVID vaccine for children 12 to 15 years old on May 10, 2021. https://www.fda.gov/news-

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individuals. *Id.* The FDA approval of Comirnaty referenced testing involving approximately 40,000 individuals who received the vaccine or a placebo and reported that the vaccine was 91 percent effective in preventing COVID. *Id.*

The Pfizer-BioNTech vaccine authorized for EUA use and Comirnaty were identical vaccines, and accordingly a fully FDA-authorized vaccine was publicly available to individuals 16 or older as of August 23, 2021. *See Johnson v. Brown*, 567 F. Supp. 3d 1230, 1247 (D. Or. 2021) ("The vaccines themselves … have the same formulation, are interchangeable, and are identical for safety and effectiveness" and although considered "legally distinct" vaccines, they are not "so different that a vaccine mandate involving one versus the other, when one has been FDA-approved as safe and effective, is a forced medical 'experiment'[.]"); *see also id.* (plaintiffs challenging Oregon vaccination requirement "acknowledge that Comirnaty is not experimental" given FDA approval); *Legaretta v. Macias*, 603 F. Supp. 3d 1050, 1061 (D.N.M. 2022) ("The Pfizer COVID-19 vaccine has been fully approved by the FDA, and thus is not 'an experimental drug unlicensed for its present use.'") (quoting *Doe v. Rumsfeld*, 297 F. Supp. 2d 119, 123 (D.D.C. 2003)).

Shriners Children's Institutes System-Wide Vaccine Requirement. With the FDA fully approving Comirnaty as a COVID vaccine, Shriners Children's on September 14, 2021, instituted a system-wide vaccination

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requirement for its employees. Compl. ¶ 277. Employees had to receive their first vaccination dose by October 11, 2021, and be fully vaccinated by December 6, 2021. *Id.* ¶ 278. Shriners Children's stated purpose in enacting the requirement was "[t]o help protect Shriners Hospitals for Children (SHC) pediatric patients from COVID-19 disease" and "[t]o help protect employed staff, non-employees, volunteers, patients and families at SHC hospitals/facilities from COVID-19 disease and to help prevent the unnecessary spread of the COVID-19 virus between employees, non-employees, patients and families." Compl. Ex. E (ECF No. 1-5).[4] Shriners Children's permitted employees to seek medical or religious exemptions from the policy. *Id.* Shriners Children's policy further stated that "[a]ny local, regional, and state vaccination standards or mandates issued by applicable public health authorities supersede this policy." *Id.*

Importantly, at the time of Shriners Children's vaccination requirement, (1) a fully FDA-approved COVID vaccine was available in Comirnaty (the Pfizer-BioNTech vaccine) and (2) Shriners Children's youngest patients were still ineligible to be vaccinated against COVID, as the FDA did not approve emergency use of the Pfizer-BioNTech vaccine in children 5 to 11 years old until October 29, 2021. https://www.fda.gov/news-events/press-announcements/fda-authorizes-pfizer-biontech-covid-19-vaccine-emergency-use-children-5-through-11-years-age. With the emergence of the delta variant, COVID cases in children

[4] Shriners Hospitals for Children's full vaccination policy was incorporated by reference into Plaintiffs' Complaint and is properly considered on this Motion.

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additionally were surging in August and September 2021; data from the American Academy of Pediatrics and Children's Hospital Association shows that COVID cases in children increased from 8,447 nationally for the week ending June 17, 2021, to 251,781 for the week ending September 9, 2021 (the highest number of the entire pandemic to that point). https://downloads.aap.org/AAP/PDF/AAP%20COVID-19%20State%20Data%20Report%205.11.23%20FINAL.pdf; *see also supra* n.2 (matters of public record not subject to reasonable dispute and whose accuracy cannot reasonably be questioned are properly judicially noticeable when considering a motion to dismiss).

III. ARGUMENT

A. Motion to Dismiss Standard.

Shriners Children's move for dismissal pursuant to Federal Rule of Civil Procedure 12(b)(6). A motion to dismiss for failure to state a claim "tests the legal sufficiency" of Plaintiffs' claims. *Navarro v. Block*, 250 F.3d 729, 732 (9th Cir. 2001). A complaint should be dismissed under Rule 12(b)(6) for failure to state a claim on which relief can be granted, unless it "contain[s] ***sufficient factual matter***, accepted as true, to 'state a claim to relief that is plausible on its face.'" *Ashcroft v. Iqbal*, 556 U.S. 662, 678 (2009) (quoting *Bell Atlantic v. Twombly*, 550 U.S. 544, 570 (2007)) (emphasis added). While Plaintiffs' factual allegations are accepted as true and construed in the light most favorable to them, Plaintiffs cannot rely on "conclusory allegations of law and unwarranted inferences … to defeat a motion to dismiss for failure to state a claim." *In re Stac Elecs. Sec.*

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Litig., 89 F.3d 1399, 1403 (9th Cir. 1996). Nor can a plaintiff rely on "labels and conclusions, and a formulaic recitation of the elements." *Twombly*, 559 U.S. at 555. The Court further "may disregard allegations that are contradicted by matters properly subject to judicial notice or by exhibit," as well as "conclusory allegations and arguments which are not supported by reasonable deductions and inferences." *Donovan v. Biden*, 603 F. Supp. 3d 975, 981 (E.D. Wash. 2022) (citing *Sprewell v. Golden State Warriors*, 266 F.3d 979, 988 (9th Cir. 2001)).

B. The Court Should Dismiss Each of Plaintiffs' Claims Against Shriners Children's.

As a threshold issue, the Court should dismiss Plaintiffs' six claims brought under Section 1983 because the Shriners Children's Defendants were not and are not state actors, and even if they had been, the individually named Shriners Children's Defendants would be entitled to qualified immunity. Further, even if the Court concludes that Shriners Children's vaccination requirement constitutes state action for which Plaintiffs can pursue Section 1983 claims, the Court should nevertheless dismiss each of Plaintiffs' claims against Shriners Children's.

1. Each of Plaintiffs' Six Section 1983 Claims Fails Because None of the Shriners Children's Defendants are State Actors, and All Those Individually Named Would Be Entitled to Qualified Immunity.

Each of Plaintiffs' six claims under Section 1983 fails because the Shriners Children's Defendants are not state actors and, even if they were, the individually named defendants would have qualified immunity. *See* Compl. ¶¶ 315-384 (Counts 1-6). Courts "start with the presumption that conduct by private actors is

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not state action" and the plaintiff "bears the burden of establishing that [d]efendants were state actors." *Florer v. Congregation Pidyon Shevuyim, N.A.*, 639 F.3d 916, 922 (9th Cir. 2011). The Shriners Children's Defendants are unquestionably "private parties," as Plaintiffs acknowledge. Compl. ¶ 276. If, as here, the plaintiff cannot meet the burden of showing the Shriners Children's Defendants are state actors, the claim must be dismissed. *See, e.g.*, *Prophet v. Dunham*, 358 F. App'x 890, 891 (9th Cir. 2009) (affirming dismissal of Section 1983 claim "because the complaint pleaded no facts suggesting that [defendant] was a state actor").

And even if Plaintiffs could establish that the Shriners Children's Defendants are state actors, their claims still fail because the individually named defendants have qualified immunity given that they had no notice that a vaccination requirement could or would violate federal law (and the U.S. Supreme Court indeed subsequently concluded that a federal requirement for healthcare workers is lawful). In analyzing qualified immunity, federal courts employ a two-prong analysis: whether the facts show the state actor's "conduct violated a constitutional right" and "whether the right was clearly established" at the time of the alleged unlawful action. *See Saucier v. Katz*, 533 U.S. 194, 201 (2001), *overruled in part on other grounds by Pearson v. Callahan*, 555 U.S. 223, 236 (2009) (overruling *Saucier* by holding that courts may begin and end their analysis with ***either*** of the two prongs). Plaintiffs have shown neither prong.

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Moreover, to determine the second prong of the qualified immunity test, whether a right is clearly established, "the reviewing court must consider whether a reasonable [official] would recognize that his or her conduct violate[d] that right under the circumstances faced, and in light of the law that existed at that time." *Kennedy v. City of Ridgefield*, 439 F.3d 1055, 1065 (9th Cir. 2006). If a state actor did not have sufficient notice that their alleged conduct would violate federal law, they are entitled to qualified immunity. *See, e.g.*, *Jones v. Cnty. of Los Angeles*, 722 F. App'x 634, 637–38 (9th Cir. 2018) ("[T]here is no Ninth Circuit or Supreme Court case law that could have provided Dr. Wang with sufficiently fair notice that her purported seizure of G.J. in order to investigate plausible child abuse, under the circumstances in this case, would violate federal constitutional law. Without such fair notice, we hold that Dr. Wang is entitled to qualified immunity."). Here, the Shriners Children's Defendants' actions were consistent with—not in conflict with—federal law, as evidenced by the U.S. Supreme Court's support for the Department of Health and Human Services Secretary's ("HHS Secretary") federal vaccination requirement for healthcare workers. They are entitled to qualified immunity in the unlikely event they are considered state actors. *Biden*, 595 U.S. at 95 ("Vaccination requirements are a common feature of the provision of healthcare in America" and federal COVID vaccination requirement "is a straightforward and predictable example of the 'health and safety' regulations that Congress has authorized the Secretary to impose.").

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a. None of the Shriners Children's Defendants are State Actors.

None of the Shriners Children's Defendants—entities or individuals—are state actors. Plaintiffs acknowledge that Defendant Shriners Hospitals for Children, Inc., "is a charitable 501C(3) non-profit corporation," and that Defendant Shriners Hospital for Children – Spokane is "a subsidiary of Shriners Hospitals for Children." Compl. ¶¶ 19.1, 19.2. Each of the individually named Shriners Children's Defendants was identified as a "Policymaker of Shriners." *Id.* ¶¶ 19.3-19.9. It is self-evident that a private nonprofit hospital, along with its officers and employees, do not qualify as state actors when they make corporate decisions, including by setting internal policies intended to protect their staff and patients and their families. Indeed, Plaintiffs acknowledge that the Shriners Children's Defendants "at other times and in other circumstances, are private parties," who would not be subject to Section 1983 claims. *Id.* ¶ 276.

Plaintiffs attempt (and fail) to summarily overcome this hurdle by alleging that the Shriners Children's Defendants are in fact state actors based on four different theories—the public function test, the state compulsion test, the symbiotic relationship test, and the customs test. *Id.* Caselaw, and common sense, show that none of these theories applies to Plaintiffs here.

First, the "public function test" states that "when private individuals or groups are endowed by the State with powers or functions governmental in nature, they become agencies or instrumentalities of the State and subject to its constitutional limitations." *Evans v. Newton*, 382 U.S. 296, 299 (1966). To

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satisfy the public function test, the function at issue must be both traditionally and ***exclusively*** governmental. *Rendell-Baker v. Kohn*, 457 U.S. 830, 842 (1982). While Plaintiffs allege that "[t]he CDC COVID-19 Vaccination Program was exclusively a government function for the public," Compl. ¶ 276 n.94, the Court need not heed this legal conclusion as true on a motion to dismiss, *Twombly*, 550 U.S. at 555. Participation in, or even facilitation of, a government-funded vaccination program does not constitute a public function that converts an otherwise private entity into a state actor. *See, e.g.*, *Finkbeiner v. Geisinger Clinic*, 623 F. Supp. 3d 458, 467 (M.D. Pa. 2022) (private hospital implementing COVID-19 vaccine mandate was not a state actor because it did not perform a function traditionally and exclusively reserved to the state); *accord Together Employees v. Mass. Gen. Brigham Inc.,* 19 F.4th 1, 3 (1st Cir. 2021) (private hospital was not a state actor governed by the First Amendment and, thus, its policy requiring all employees to be vaccinated against COVID-19 did not give rise to viable Section 1983 claim). Plaintiffs have not shown—and cannot show—otherwise.

If Plaintiffs' contention were true, every hospital that administers governmental programs in exchange for government funding would be a "state actor," which is plainly not the case. *See Yesilevsky v. Redmond*, 2020 WL 4370957, at *8 (C.D. Cal. Apr. 1, 2020) (holding that "state regulation or receipt of state funds alone does not turn the Hospital into a state actor"); *cf. Rendell-Baker*, 457 U.S. at 842 (where the "State intends to provide services . . . at public

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expense[,] [t]hat legislative policy choice in no way makes these services the exclusive province of the State" sufficient to make the provider of said services a state actor). This theory fails to convert Shriners Children's into a state actor.

Second, under the "public compulsion test," private action may be deemed state action when the government has "exercised coercive power or has provided such significant encouragement, either overt or covert, that the [private actor's] choice must in law be deemed to be that of the State." *Blum v. Yaretsky*, 457 U.S. 991, 1004 (1982). The implementation of a generally applicable law does not constitute compulsion under this test, as the Ninth Circuit has held:

> In summary, Supreme Court precedent does not suggest that governmental compulsion in the form of a generally applicable law, without more, is sufficient to deem a private entity a governmental actor. Instead, the plaintiff must establish some other nexus sufficient to make it fair to attribute liability to the private entity. Typically, the nexus has consisted of participation by the state in an action ostensibly taken by the private entity, through conspiratorial agreement …, official cooperation with the private entity to achieve the private entity's goal …, or enforcement and ratification of the private entity's chosen action ….

Sutton v. Providence St. Joseph Med. Ctr., 192 F.3d 826, 841 (9th Cir. 1999) (internal citations omitted).

Here, Plaintiffs allege without any explanation that Defendant Shriners Children's Spokane "is a state actor under this test pursuant to *Blum*." Compl. ¶ 276 n.95. But they do not allege the nature of the state's alleged compulsion at all, and specifically they do not—and cannot—allege any "nexus sufficient to make it fair to attribute liability to" Shriners Children's, including any participation by or cooperation with the state in its implementation of its vaccine

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policy, or any state enforcement and ratification of that policy. *Sutton*, 192 F.3d at 841. Plaintiffs' presumptive position that the state's Proclamation amounts to compulsion under this test does not meet this requirement, because the Proclamation amounts to a generally applicable law that applies to all employers. As the *Sutton* court observed, "[t]o accept Plaintiff's argument would be to convert every employer—whether it has one employee or 1,000 employees—into a governmental actor every time it complies with a presumptively valid, generally applicable law, such as an environmental standard or a tax-withholding scheme." *Sutton*, 192 F.3d at 838. This theory also fails to convert Shriners Children's into a state actor.

Third, the "symbiotic relationship test" asks "whether the government has so far insinuated itself into a position of interdependence with a private entity that the private entity must be recognized as a joint participant in the challenged activity." *Brunette v. Humane Soc'y of Ventura Cnty.*, 294 F.3d 1205, 1210 (9th Cir. 2002). A private entity may be considered a state actor "only if its particular actions are 'inextricably intertwined' with those of the government," of which "significant financial integration" between the private entity and the state is a hallmark. *Id.* at 1211, 1213; *see also Ponce v. Basketball Fed'n of Puerto Rico*, 760 F.2d 375, 382 (1st Cir. 1985) ("In *Rendell-Baker v. Kohn*, the United States Supreme Court stressed that the key factor in determining the existence of a symbiotic relationship is whether the *state* profited from the discriminatory activity.") (emphasis added, internal citations omitted). While Plaintiffs allege

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that when "a private entity . . . confers significant financial benefits indispensable to the government's 'financial success,' then a symbiotic relationship may exist," Compl. ¶ 276 n.96, Plaintiffs never actually allege that Shriners Children's administration of COVID-19 vaccines (or its vaccination requirement for employees) conferred ***any*** financial benefit—let alone "significant financial benefits"—on either Shriners Children's *or* the government sufficient to make the two inextricably intertwined as needed to satisfy the symbiotic relationship test. *See generally* Compl.; *Ponce*, 760 F.2d at 382. Yet again, this theory fails to convert Shriners Children's into a state actor.

Finally, the "state-enforced customs test" requires action under direction of policy or custom having the force of law advanced by the state. *See Adickes v. S.H. Kress & Co.*, 398 U.S. 144, 163-69 (1970). Bewilderingly, Plaintiffs allege that Shriners Children's participated in *developing* the "state custom" about which Plaintiffs complain. *See* Compl. ¶ 276 n.97. A custom *developed* in part by Shriners Children's alleged actions cannot simultaneously have compelled those same actions by "having the force of law." *Adickes*, 398 U.S. at 163. Here too, this theory fails to convert Shriners Children's into a state actor.

Ultimately, no matter the test, "[t]he ultimate issue in determining whether a person is subject to suit under Section 1983 is [asking,] . . . is the alleged infringement of federal rights fairly attributable to the [government]?" *Sutton*, 192 F.3d at 835 (quoting *Rendell-Baker*, 457 U.S. at 838). Even construing the

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facts alleged by Plaintiffs as true, the answer to this question is plainly "no" for a private hospital instituting a vaccination requirement for its workforce.

The Shriners Children's Defendants are private entities and individuals and thus cannot be the subjects of Section 1983 claims. No theory supports Plaintiffs' legal conclusions that Shriners Children's in any way functioned as state actors. Each of Plaintiffs' six Section 1983 claims should accordingly be dismissed.

b. Even if Any Individually Named Shriners Children's Defendant Were a State Actor, They Would Be Entitled to Qualified Immunity.

Even if Plaintiffs established that any Shriners Children's Defendant was a state actor—which they cannot—Plaintiffs' Section 1983 claims would still fail against the individually named Shriners Children's Defendants because the latter would be entitled to qualified immunity. As in *Jones*, at the time Shriners Children's instituted its employee vaccination requirement, no authority suggested or provided notice that such a requirement would or even could violate federal law. 722 F. App'x 634, 637-38. Indeed, the U.S. Supreme Court subsequently held that a federal healthcare employee vaccination requirement issued by the HHS Secretary for hospitals receiving Medicare and Medicaid funding was "consistent with the fundamental principle of the medical profession" to "first, do no harm." *Biden*, 595 U.S. at 93. Without "fair notice" that their conduct could violate federal law (which it did not), the individually named Shriners Children's Defendants are entitled to qualified immunity with respect to Plaintiffs' Section 1983 claims.

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2. Each of Plaintiffs' Claims Fails Regardless of Whether the Shriners Children's Defendants are Considered State Actors.

In addition to Plaintiffs' inability to assert Section 1983 claims against the Shriners Children's Defendants as private entities and individuals, Plaintiffs' claims fail for additional reasons.

a. Plaintiffs Were Not Subjected to "Investigational Drug Use" and No Private Right of Action Under the Statutes, Regulations, and Other Sources Referenced Supports a Section 1983 Claim.

Plaintiffs' claim that they were subjected to investigational drug use (Count 1) fails for two predicate reasons, and further because there is no private right of action for Plaintiffs to assert a Section 1983 claim under the statutes, regulations, reports, and treaties referenced.

(1) Plaintiffs Never Received a COVID Vaccine and Such Vaccines Were Not "Investigational" at the Time of Shriners Children's Policy.

While Plaintiffs assert that they were "subjected without [their] free consent to medical or scientific experimentation," Compl. ¶ 324, Plaintiffs do not claim that they ever received a COVID vaccine. They were never "subjected" to anything. Shriners Children's enacted a COVID vaccination requirement broadly applicable to their workforce; they did not force Plaintiffs to serve as "human subject[s]" for research. Compl. ¶ 319. Indeed, a Texas federal district court rejected a similar characterization of an employee vaccination requirement as somehow a forced human trial in dismissing similar claims against Houston Methodist Hospital:

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> The hospital's employees are not participants in a human trial. They are licensed doctors, nurses, medical technicians, and staff members. The hospital has not applied to test the COVID-19 vaccines on its employees, it has not been approved by an institutional review board, and it has not been certified to proceed with clinical trials.

Bridges v. Houston Methodist Hosp., 543 F. Supp. 3d 525, 527-28 (S.D. Tex. 2021).

The court in *Bridges* added that Houston Methodist's vaccination requirement was not "coercion" upon its employees (as Plaintiffs similarly allege here). "Methodist is trying to do their business of saving lives without giving them the COVID-19 virus. It is a choice made to keep staff, patients, and their families safer. [Plaintiff] can freely choose to accept or refuse a COVID-19 vaccine; however, if she refuses, she will simply need to work somewhere else." *Id.*; *see also Johnson*, 567 F. Supp. 3d at 1256-57 (Oregon vaccination requirement "presents Plaintiffs with a difficult choice, but it is nevertheless a choice. Plaintiffs may either get the vaccine, apply for an exception, or look for employment elsewhere."); *id.* at 1248 ("Plaintiffs here do not contend that they are being forced to be part of the *clinical trials* for the Pfizer-BioNTech Vaccine or that they are being forcibly injected while being physically held against their will.") (emphasis in original). Here, Plaintiffs were not "subjected" to anything (as they do not claim to have received COVID vaccines), they were not forced participants in a human trial, and they were not "coerced" when they chose not to receive the vaccine. On this basis alone, their claim fails.

In addition, Plaintiffs' claim also fails given that the COVID vaccine at issue was not "experiment[al]" or "investigational" as they allege. Compl. ¶¶

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323-325. At the time Shriners Children's instituted their employee vaccine requirement, the FDA had fully approved Comirnaty as a COVID vaccine for individuals 16 and older. Comirnaty had the "same formulation" and was "interchangeable" with the Pfizer-BioNTech vaccine authorized for emergency use. *Johnson*, 567 F. Supp. 3d at 1252. Accordingly, a fully approved vaccine was available for Shriners Children's employees. *See id.* at 1247 ("Plaintiffs acknowledge, Comirnaty is not experimental."); *id.* (the vaccines are not "so different that a vaccine mandate involving one versus the other, when one has been FDA-approved as safe and effective, is a forced medical 'experiment'"); *Legaretta*, 603 F. Supp. 3d at 1061 ("The Pfizer COVID-19 vaccine has been fully approved by the FDA and thus is not 'an experimental drug unlicensed for its present use.'") (quoting *Rumsfeld*, 297 F. Supp. 2d at 123); *Valdez v. Grisham*, 559 F. Supp.3d 1161, 1171 (D.N.M. 2021) (challenges to emergency use vaccines "are not applicable to the administration of the Pfizer vaccine to individuals 16 years of age and older" given FDA approval of Comirnaty). Plaintiffs claim they "make no assertions regarding whether it is lawful for a public or private entity to mandate a ***licensed*** vaccine." Compl. ¶¶ 20, 119 (emphasis in original). But that is precisely what is at issue here—Shriners Children's vaccination requirement followed FDA approval of Comirnaty as a COVID vaccine. Plaintiffs accordingly were not required to receive "experimental" or "investigational" vaccines to comply with Shriners Children's policy, and their claim fails on this basis as well.

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(2) No Private Right of Action Exists Under Section 1983 for Any of the Statutes, Regulations, or Other Sources Plaintiffs Reference for Their Claim.

Even more, none of the statutes, regulations, reports, and treaties Plaintiffs reference in support of their claim provides a private right of action under Section 1983. A plaintiff "may invoke [a Section 1983] cause of action against any other person who, acting 'under color of' state law, has deprived them of 'any rights, privileges, or immunities secured by the Constitution and laws' of the United States." *Health & Hosp. Corp. of Marion Cnty. v. Talevski*, 599 U.S. 166, 175 (2023) (quoting 42 U.S.C. § 1983). But the Supreme Court has recognized a "demanding bar" before finding that a statute creates Section 1983-enforceable rights, and the statutory provision(s) at issue "must *unambiguously* confer individual federal rights." *Id.* at 180 (citing *Gonzaga Univ. v. Doe*, 536 U.S. 273, 280 (2002)) (emphasis in original). In particular, to find enforceable Section 1983 rights, "it must be determined that 'Congress intended to create a federal right' *for* the identified class, not merely that the plaintiffs fall 'within the general zone of interest that the statute is intended to protect.'" *Id.* at 183 (quoting *Gonzaga*, 536 U.S. at 283) (emphasis in original).

Plaintiffs cite two federal statutes related to their claim. The first, 21 U.S.C. § 360bbb-3 (the "Emergency Use Authorization Act" or "EUA Act"), governs the authorization of emergency use drugs, devices, or biological products by the HHS Secretary during a public health or other national emergency. Compl. ¶¶ 317-318. The statute outlines the criteria, scope, and conditions

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associated with the authorization of an emergency use drug or other product, including consideration of the known and potential benefits of the drug/product and the known or potential risks. *Id.* § 360bbb-3(c), (d), (e).

Courts that have considered the EUA Act have ***uniformly*** concluded that it provides no private right of action to assert Section 1983 claims. "This statute gives the [HHS Secretary] power to act in an emergency. It does not confer a private right to sue." *Smith v. Terminix Pest Control, Inc.*, 2023 WL 3569127, at *2 (E.D. La. May 19, 2023) (case brought by same plaintiffs' counsel as here); *Bridges*, 543 F. Supp. 3d at 527 (EUA Act "confers certain powers and responsibilities to the [HHS Secretary] in an emergency. It neither expands nor restricts the responsibilities of private employers; in fact, it does not apply at all to private employers like the hospital in this case. It does not confer a private opportunity to sue the government, employer, or worker."); *Kiss v. Best Buy Stores*, 2022 WL 17480936, at *8 (D. Or. Dec. 6, 2022) ("courts have consistently held that the Emergency Use Authorization Act provides no private right of action") (collecting cases). Accordingly, the EUA Act provides no basis for Plaintiffs to assert a claim under Section 1983 for being subjected to supposed "investigational drug use."

Similarly, 10 U.S.C. section 980, the other federal statute that Plaintiffs cite, does not provide a basis for a claim against Defendants. Compl. ¶ 316. Section 980 addresses limitations on research ***conducted by the Department of Defense*** on human subjects. *See Doe v. Sullivan*, 938 F.2d 1370, 1374-75 (D.C.

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Cir. 1991) (referencing claim under Section 980 by servicemember stationed in Middle East concerning DOD's use of alleged unapproved investigational drugs without obtaining informed consent). Here, Plaintiffs do not allege to be servicemembers and they are not bringing a claim against the Department of Defense. Accordingly, 10 U.S.C. section 980 cannot be the basis of a claim against Defendants.

Plaintiffs also cite 45 C.F.R. § 46.116, an HHS regulation concerning informed consent for human research subjects. Compl. ¶¶ 319-321. Again, Plaintiffs do not claim they ever received a COVID vaccine, and they were not human subjects in a research program. Regardless, the Ninth Circuit has held that "agency regulations cannot independently create rights enforceable through § 1983." *Save Our Valley v. Sound Transit*, 335 F.3d 932, 939 (9th Cir. 2003); *see also id.* at 943 ("Plaintiffs suing under § 1983 must demonstrate that a *statute*—not a regulation—confers an individual right.") (emphasis in original); *Courtney v. Housing Auth. of Cnty. of Kings*, 2021 WL 168285, at *5 (E.D. Cal. Jan. 19, 2021) ("It is not clear what cause of action Plaintiff is attempting to assert by citing to these regulations, as the regulations, on their own, do not provide a private right of action."). Citing 45 C.F.R. § 46.115 provides no grounds for a claim under Section 1983.

Plaintiffs lastly cite other sources that do not even rise to the level of a federal regulation, let alone a statute as necessary to create potential Section 1983-enforceable rights. The 1979 Belmont Report (Compl. ¶ 322) presented

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findings and conclusions regarding medical research and informed consent to Congress, but it does not create a private right of action. *See Kriley v. Nw. Mem'l Healthcare*, 2023 WL 371643, at *2 (7th Cir. Jan. 24, 2023) (Belmont Report is a "statement[] of principles"). The International Covenant on Civil and Political Rights (ICCPR) Treaty (Compl. ¶ 324) also provides no basis for a private right of action. The ICCPR Treaty "do[es] not create rights enforceable in federal courts." *Kriley*, 2023 WL 371643, at *2 (citing *Sosa v. Alvarez-Machain*, 542 U.S. 692, 728, 734 (2004)); *see also Marcus v. Centers for Disease Control & Prevention*, 2023 WL 3044614, at *13 (C.D. Cal. Feb. 21, 2023) (same). In sum, none of the statutes, regulations, reports, and treaties referenced by Plaintiffs supports a Section 1983 claim, warranting dismissal of Plaintiffs' claim.

b. Plaintiffs' Due Process Claim Fails, as Shriners Children's Satisfied Procedural Requirements and Vaccine Requirements Uniformly Survive Rational Basis Review.

As pleaded, Plaintiffs suggest that Shriners Children's violated both their procedural and substantive due process rights under the Fourteenth Amendment. Compl. ¶¶ 334, 338. The Court should dismiss Plaintiffs' due process claim (Count 3) on both grounds.

(1) Shriners Children's Complied with Procedural Due Process Requirements Recognized by this Court in Enacting its Vaccination Policy.

To the extent that Plaintiffs focus their due process claim on procedural grounds—referencing "the opportunity to be heard" and "the Constitutional right 'to present [their] case and have its merits fairly judged,'" Compl. ¶¶ 335, 337—

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the Court should dismiss their claim for the reasons previously cited by this Court in *Bacon*, 2021 WL 5183059, and *Wise v. Inslee*, 2022 WL 1243662 (E.D. Wash. April 27, 2022).

In *Bacon*, this Court concluded that city employees were not entitled to greater process than the notice of the City of Spokane's vaccination requirement that was provided. 2021 WL 5183059, at *2-3. "[W]hen a policy is generally applicable, employees are not 'entitled to process above and beyond the notice provided by the enactment and publication' of the policy itself." *Id.* at *3 (quoting *Harris v. Univ. of Mass.*, 2021 WL 3848012, at *5 (D. Mass. Aug. 27, 2021)). As the Court added: "District courts around the country have applied this principle to employer-issued vaccine mandates during the COVID-19 pandemic, finding employees are not entitled to greater service than what is provided by enactment of the mandates themselves." *Id.* The Court concluded that in enacting a "generally applicable" vaccination requirement, the City was not required to provide employees more process "beyond what was provided by enacting" the requirement, but the City nevertheless went "above and beyond" in providing advance notice of the requirement and providing the opportunity for employees to submit information and participate in hearings concerning potential accommodations. *Id.* "Simply disagreeing with the available accommodations is insufficient to challenge the constitutionality of the City's vaccination requirement" on procedural due process grounds. *Id.*

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This Court followed the same analysis in dismissing a procedural due process claim brought by employees challenging the State's vaccination requirement in *Wise*. 2022 WL 1243662, at *5. And this analysis is equally applicable to Plaintiffs' claims here: Shriners Children's enacted a generally applicable vaccination requirement for its employees, then provided those employees with the opportunity to submit information regarding potential exemptions from the vaccination requirement. Compl. Ex. E (ECF No. 1-5). At least four of the plaintiffs acknowledge seeking medical exemptions under the procedure that Shriners Children's established to consider such exemptions. Compl. ¶ 300. Accordingly, Shriners Children's complied with procedural due process requirements as recognized by this Court for establishing a vaccination requirement generally applicable to its employees.

(2) Plaintiffs' Substantive Due Process Claim Fails as Shriners Children's Vaccine Requirement Does Not Implicate a Fundamental Right and Easily Survives Rational Basis Review.

To the extent Plaintiffs assert a substantive due process claim (referencing a vaccine requirement as "not a legitimate exercise of police power of the State," Compl. ¶ 336), Plaintiffs' claim fails as well, as vaccine requirements do not implicate a fundamental right, and vaccination requirements (particularly for healthcare workers) easily survive rational basis review.

"Substantive due process protects individuals from arbitrary deprivation of their liberty by government." *Sylvia Landfield Tr. v. City of Los Angeles*, 729 F.3d 1189, 1195 (9th Cir. 2013). "The Due Process Clause 'provides heightened

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protection against government interference with certain fundamental rights and liberty interests.'" *Williams*, 567 F. Supp. 3d at 1224 (quoting *Washington v. Glucksberg*, 521 U.S. 702, 720 (1997)). "The Supreme Court has established 'a threshold requirement—that a challenged state action implicate a fundamental right—before requiring more than a reasonable relation to a legitimate state interest to justify the action.'" *Id.* at 1224 (quoting *Glucksberg*, 521 U.S. at 722).

But courts have found that vaccination requirements do not implicate fundamental rights, dating to the U.S. Supreme Court's decision in *Jacobson v. Massachusetts*, 197 U.S. 11 (1905). In *Jacobson*, the Court upheld a smallpox vaccination requirement imposed by the city of Cambridge, Mass. (and enacted pursuant to Massachusetts state law), that provided criminal sanctions for individuals who refused to comply. *Id.* at 12-13, 39. The Court held that Cambridge's requirement was a valid exercise of police power by the State and that the requirement did not infringe on the unvaccinated plaintiff's liberty interests. *Id.* at 25-26. The Court expressed that there was no "privilege of a minority" to reside in a city or town where they enjoy the general protection of an organized local government while nevertheless "defy[ing] the will of its constituted authorities" trying to ensure the welfare and safety of the community. *Id.* at 37-38. As the Court concluded in *Jacobson*:

> We are unwilling to hold it to be an element in the liberty secured by the Constitution of the United States that one person, or a minority of persons, residing in any community and enjoying the benefits of its local government, should have the power thus to dominate the majority when supported in their action by the authority of the state. … The safety and the health of the people of Massachusetts are, in

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> the first instance, for that commonwealth to guard and protect. They are matters that do not ordinarily concern the national government. So far as they can be reached by any government, they depend, primarily, upon such action as the state, in its wisdom, may take; and we do not perceive that this legislation has invaded any right secured by the Federal Constitution.

Id. at 38.

In considering *Jacobson* and a challenge to the University of California's COVID vaccination requirement, the Ninth Circuit concluded that no "historical example" exists "establish[ing] a 'fundamental right' to be free from a vaccine mandate at a workplace." *Kheriaty v. Regents of the Univ. of Calif.*, 2022 WL 17175070, at *1 (9th Cir. Nov. 23, 2022). As no fundamental right is implicated by a vaccine requirement, the Ninth Circuit held that rational basis review should apply to the COVID vaccine policy challenge. *Id.*; *see also Williams*, 567 F. Supp. 3d at 1226 ("In the context of COVID-19, courts across the country have concluded that *Jacobson* established that there is no fundamental right to refuse vaccination" and "[c]ourts have also consistently held that, following *Jacobson*, challenges to vaccination mandates are properly considered under rational basis review.") (collecting cases); *Burcham v. City of Los Angeles*, 562 F. Supp. 3d 694, 706 (C.D. Cal. 2022) ("The Supreme Court has long rejected a fundamental right to refuse vaccination.") (citing *Jacobson*).

As courts thus have routinely recognized, Shriners Children's vaccination requirement—applicable to all its employees, in accordance with state mandates, and intended to protect its patients and workers from COVID—satisfies rational basis review. Again, in *Biden*, the U.S. Supreme Court emphasized that

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"[v]accination requirements are a common feature of the provision of healthcare in America," such as requirements that healthcare workers be vaccinated for hepatitis, influenza, and measles, mumps, and rubella. 595 U.S. at 95. And the "overwhelming[]" support for the HHS Secretary's COVID vaccination requirement from healthcare workers and public-health organizations "suggests that a vaccination requirement under these circumstances is a straightforward and predictable example of the 'health and safety' regulations that Congress has authorized the Secretary to impose." *Id.*[5]

"Under rational basis review, state conduct is presumed valid and will be upheld so long as it is 'rationally related to a legitimate state interest.'" *Williams*, 567 F. Supp.3d at 1226 (quoting *City of Cleburne v. Cleburne Living Ctr.*, 473 U.S. 432, 440 (1985)). Shriners Children's vaccination requirement, intended to protect its staff and patients and their families from the transmission of COVID (particularly for its youngest patients, ineligible at the time to receive a COVID vaccine, even as weekly COVID case rates for children surged nearly 3,000 percent between June and September 2021 due to the spread of the delta variant,

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[5] In *Roman Catholic Diocese of Brooklyn v. Cuomo*, 141 S. Ct. 63 (2020), the Court expressed that "[s]temming the spread of COVID-19 is unquestionably a compelling [government] interest," *id.* at 67, and suggested that the vaccination requirement in *Jacobson* "easily survived rational basis review," *id.* at 71 (Gorsuch, J. concurring).

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see supra Section II at 11), easily survives the deferential rational basis review standard. *See Wise*, 2022 WL 1243662, at *5-6 (dismissing substantive due process claim as state vaccination requirement "survives rational basis review because it is rationally related to the State's legitimate interest in stemming the spread of COVID-19 among its employees who come into close contact with vulnerable members of the population"); *Johnson v. Brown*, 614 F. Supp.3d 776, 782 (D. Or. 2022) ("The decision to require vaccination among critical populations, such as healthcare workers and providers and education workers and volunteers, is a rational way to further the State's interest in protecting everyone's health and safety during the COVID-19 pandemic."); *Burcham*, 562 F. Supp. 3d at 707 (vaccination requirement that followed state and federal guidance was "rationally related to controlling the spread of COVID-19"); *Antunes v. Rector & Visitors of Univ. of Va.*, 627 F. Supp. 3d 553, 565 (W.D. Va. 2022) ("Based on the risk of COVID-19 contagion in a health care setting, a vaccination requirement for healthcare staff is rationally related to the legitimate state interest of preventing the spread of COVID-19."); *Does 1-6 v. Mills*, 566 F. Supp. 3d 34, 52 (D. Me. 2021) ("Stopping the spread of COVID-19 in Maine, and specifically stemming outbreaks in designated healthcare facilities to protect patients and healthcare workers, is a legitimate government interest" and the challenged vaccine requirement for healthcare workers "is rationally related to this interest.").

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Shriners Children's vaccination requirement should be evaluated under the same rational basis standard, as it complied with Proclamation 21-14, supported the hospital's express objectives of "protect[ing] employed staff, non-employees, volunteers, patients and families at SHC hospitals/facilities from COVID-19 disease and … help[ing] prevent the unnecessary spread of the COVID-19 virus between employees, non-employees, patients and families," and was instituted at a time of surging COVID case rates for children. Compl. Ex. E (ECF No. 1-5). Under this standard, Plaintiffs' substantive due process challenge fails, and the Court should dismiss their claim.

c. The Court Should Dismiss Plaintiffs' Equal Protection Claim on Similar Grounds.

Similar to Plaintiffs' substantive due process claim, the Court should dismiss Plaintiffs' equal protection claim (Count 2), as Plaintiffs are not members of a protected class, Shriners Children's vaccine requirement does not implicate Plaintiffs' fundamental rights, and vaccination requirements uniformly survive rational basis review. "To prevail on an equal protection claim, a plaintiff must 'show that a class that is similarly situated has been treated disparately.'" *Williams*, 567 F. Supp. 3d at 1227 (quoting *Boardman v. Inslee*, 978 F.3d 1092, 1117 (9th Cir. 2020)). "'The general rule is that legislation is presumed to be valid and will be sustained if the classification drawn by the statute is rationally related to a legitimate state interest,' but heightened standards of review apply when suspect classifications like race, alienage, or national origin, are implicated

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or 'when state laws impinge on personal rights protected by the Constitution.'" *Id.* (quoting *City of Cleburne*, 473 U.S. at 440).

Plaintiffs do not claim they are members of a suspect class; rather that Shriners Children's vaccination requirement "only penalized individuals" who refused to receive a COVID vaccination. Compl. ¶ 329; *see Wolfe v. Logan*, 2023 WL 2239062, at *6 (C.D. Cal. Jan. 25, 2023) ("A classification based on COVID-19 vaccination status is not subject to heightened scrutiny under the Equal Protection Clause."); *Valdez*, 559 F. Supp. 3d at 1178 (vaccination requirements do not "target a suspect class."). As discussed, there is no constitutional right to refuse to comply with a vaccination requirement. *See supra* Section III.B.2.b.(2). Plaintiffs' equal protection claim thus must be evaluated under rational basis review. *See Williams*, 567 F. Supp. 3d at 1227-28 (concluding "courts have routinely rejected the argument that vaccine mandates will trigger heightened scrutiny under the Equal Protection Clause and have instead applied rational basis review" and "[t]he Court is fully in agreement with this growing consensus and finds that no fundamental right or suspect classification is implicated by the Oregon vaccine mandates and so rational basis review will apply to Plaintiffs' claims.").

This Court previously found that vaccination requirements similar to Shriners Children's satisfy rational basis review and do not give rise to an equal protection claim. In *Bacon*, the Court concluded that city employees' equal protection challenge to the City of Spokane's vaccination mandate failed as a

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matter of law. 2022 WL 2381021, at *3 (E.D. Wash. June 30, 2022). As the Court emphasized, "[w]hile the [City vaccination requirement] differentiates between vaccinated and unvaccinated employees, the classifications serve a legitimate government purpose, which is to slow the spread of COVID-19, and the classifications are not arbitrary or irrational." *Id.* The Court added that the City's vaccination requirement "applie[d] with equal force to all City employees," as Shriners Children's requirement did with respect to Plaintiffs and Shriners Children's other employees. *Id.*

Courts in the Ninth Circuit and nationally have consistently reached the same conclusion in dismissing equal protection claims related to COVID vaccine requirements. *See Kheriaty*, 2022 WL 17175070, at *1-2; *Williams*, 567 F. Supp. 3d at 1228 ("[T]he Court has no trouble discerning a legitimate state interest in slowing the spread of COVID-19 and the Court concludes that the vaccine mandates are rationally related to furthering that interest."); *George v. Grossmont Cmty. Coll. Dist. Bd. of Govs.*, 2022 WL 16722357, at *16 (S.D. Cal. Nov. 4, 2022) ("In concluding vaccine mandates are a rational method to reduce COVID-19 infection and the risk of serious illness, hospitalization, and death related thereto, the Court rests on all fours with its sister courts in the Ninth Circuit and other circuit courts."); *Antunes*, 627 F. Supp. 3d at 563 (dismissing equal protection claim brought by nurse challenging University of Virginia vaccination requirement; "The vaccination requirement is related to the government interest

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in preventing COVID-19 from spreading amongst UVA Health personnel and patients.").

As Plaintiffs are not members of a suspect class and vaccination requirements do not implicate fundamental rights, Shriners Children's policy is evaluated under a rational basis standard that it easily passes.

d. Plaintiffs' Unconstitutional Conditions Claim Should Be Dismissed as Courts Uniformly Have Held that Vaccine Requirements are Constitutional.

Plaintiffs' unconstitutional conditions claim (Count 5) fails for the same reason as their substantive due process and equal protection claims. The "unconstitutional conditions doctrine … vindicates the Constitution's enumerated rights by preventing the government from coercing people into giving them up." *Koontz v. St. Johns River Water Mgmt. Dist.*, 570 U.S. 595, 604 (2013). But "[a] predicate for any unconstitutional conditions claim is that the government could not have constitutionally ordered the person asserting the claim to do what it [allegedly] attempted to pressure that person into doing." *Id.* at 612.

As discussed, there is no constitutional prohibition on a vaccination requirement nor any fundamental right for an individual not to comply with such requirement, *see supra* Section III.B.2.b.(2); *Jacobson*, 197 U.S. at 37-38, let alone a vaccination requirement for healthcare workers, as Shriners Children's instituted here.

Given this, courts have "uniformly rejected claims that vaccination requirements subject plaintiffs to unconstitutional conditions." *Griner v. Biden*, 2022 WL 7501065, at *10 (D. Utah Oct. 13, 2022); *see also Legaretta*, 603 F.

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Supp. 3d at 1070-71 (county vaccine mandate "does not violate any of Plaintiffs' fundamental rights" and accordingly, "[w]ithout identifying a fundamental right, Plaintiffs' challenge to the Vaccine Mandate as an unconstitutional condition necessarily fails"); *Antunes*, 627 F. Supp. 3d at 566 (dismissing unconstitutional conditions claim when plaintiff nurse was at-will employee with no recognized constitutional benefit in continued employment); *Norris v. Stanley*, 567 F. Supp. 3d 818, 822 (W.D. Mich. 2021) (Michigan State University's "vaccine mandate does not violate any of Plaintiff's fundamental rights, so [unconstitutional conditions] argument cannot succeed"). The same reasoning supports dismissal of Plaintiffs' unconstitutional conditions claim.

e. Spending Clause Legislation is Especially Disfavored as a Basis for Section 1983 Claims.

Plaintiffs' claim that they were deprived of rights under the Spending Clause (Count 4) should be dismissed as well. As the Supreme Court recently held, Section 1983 challenges to laws that Congress enacts via its spending power should be evaluated no differently than any other statutes. *See Talevski*, 599 U.S. at 183 (holding that federal statutes have potential to create Section 1983-enforceable rights but they "do not do so as a matter of course"). For Plaintiffs here, the Supreme Court recognized that Spending Clause legislation ***is even less likely*** to give rise to rights enforceable through Section 1983 claims: "For Spending Clause legislation in particular, we have recognized that 'the typical remedy for state noncompliance with federally imposed conditions is not a private cause of action for noncompliance but rather action by the Federal Government to

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terminate funds to the State.'" *Id.* (quoting *Gonzaga*, 536 U.S. at 280); *see also id.* at 194-95 (Section 1983 actions "are the exception—not the rule—for violations of Spending Clause statutes" and "[c]ourts must tread carefully before concluding that Spending Clause statutes may be enforced through § 1983.") (Barrett, J. concurring).

The *Gonzaga* framework used to determine whether a federal statute creates a private right of action under Section 1983 is the same for spending legislation. *Id.* at 183. Plaintiffs cite the same statutes, regulations, reports, and treaties in connection with their Spending Clause claim as for their claim that they were subjected to investigational drug use. *See supra* Section III.B.2.a. As discussed, those statutes either do not provide for a private right of action enforceable through a Section 1983 claim or they are not federal statutes at all and therefore not actionable under Section 1983. The Court accordingly should dismiss Plaintiffs' Spending Clause-related claim on these grounds.

Additionally, the Court should dismiss Plaintiffs' claim given that it appears to be predicated on federal guarantees that COVID vaccines would be administered at no cost to recipients. *See* Compl. ¶¶ 343-344, 347, 349-352 (claiming this requirement "establish[es] a specific monetary entitlement to the individual"). But Plaintiffs do not allege that Shriners Children's administered a COVID vaccine to them (nor that they received the COVID vaccine at all) and were improperly required to pay for the vaccine. Plaintiffs further contend that they have a "monetary entitlement" to "obtain medical counseling" from a

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healthcare professional regarding the COVID vaccine and "not incur a fee from the professional" should they decline to receive the vaccine. *Id.* ¶ 356. Again, Plaintiffs do not allege that they received such counseling from Shriners Children's or that Shriners Children's improperly billed them for such consultation. There is ultimately neither a legal nor factual basis supporting Plaintiffs' claim.

f. The PREP Act is an Immunity Statute Providing No Basis for Plaintiffs' Claim.

Plaintiffs cannot assert a claim for supposed violation of the PREP Act, 42 U.S.C. § 247d-6d (Count 6), when they do not allege that they received a COVID vaccine, let alone suffered any injury resulting from administration of the vaccine. The PREP Act provides for immunity for "all claims for loss caused by, arising out of, relating to, or resulting from the administration to or the use by an individual of a covered countermeasure" following a declaration by the HHS Secretary of a public health emergency. 42 U.S.C. §§ 247d-6d(a)(1), (b)(1). A "covered countermeasure" includes emergency use authorized drugs and "qualified pandemic or epidemic product[s]" (such as the COVID vaccines) and a "covered person" entitled to immunity under the PREP Act includes manufacturers, distributors, and individuals who prescribe, administer, or dispense "covered countermeasures." *Id.* §§ 247d-6d(i)(1), (2). "The PREP Act protects certain covered individuals—such as pharmacies and drug manufacturers—from lawsuits during a public-health emergency"; further, "[t]he

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scope of immunity [under the PREP Act] is broad." *Estate of Maglioli v. Alliance HC Holdings LLC*, 16 F.4th 393, 400-01 (3d Cir. 2021).[6]

While providing broad immunity from claims related to the administration of covered countermeasures, the PREP Act also establishes an emergency fund to provide compensation to individuals who suffer covered injuries "directly caused by the administration or use of a covered countermeasure." 42 U.S.C. § 247d-6e(a). Otherwise, the only action that can be maintained under the PREP Act is "for an exclusive Federal cause of action against a covered person for death or serious physical injury proximately caused by willful misconduct … by such covered person." 42 U.S.C. § 247d-6d(d)(1). Such an action can only be brought in federal district court in the District of Columbia, and requires the plaintiff to submit a physician affidavit and certified medical records documenting the alleged serious physical injury or death and proximate cause connection to the covered countermeasure. 42 U.S.C. § 247d-6d(e)(1), (4)(C).

As the Third Circuit recognized, the PREP Act provides "a narrow exception" for an "exclusive" federal cause of action when an individual has

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[6] The HHS Secretary declared COVID-19 a public health emergency in accordance with the PREP Act in January 2020 and renewed that declaration at least 13 times into 2023. *See* https://www.federalregister.gov/documents/2023/05/12/2023-10216/eleventh-amendment-to-declaration-under-the-public-readiness-and-emergency-preparedness-act-for#:~:text=Extending %20the%20time%20period%20for,vaccines%20 and%20COVID%E2%80%9319%20tests.

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allegedly suffered death or serious physical injury proximately caused by willful misconduct related to a covered countermeasure. *Maglioli*, 16 F.4th at 409. Otherwise, "[f]or nearly all injuries caused by covered countermeasures falling under the Act, the [HHS] Secretary has the sole authority to administer and provide compensation from the designated fund." *Id.*; *see also Schuster v. Percheron Healthcare, Inc.*, 493 F. Supp.3d 533, 537-38 (N.D. Tex. 2021) ("The PREP Act is, at its core, an immunity statute; it creates no rights, duties, or obligations," and, "except for one narrow exception" for claims alleging willful misconduct, "PREP Act claims cannot be brought in federal court.").

Plaintiffs fail to allege an actionable claim under the PREP Act. They reference "[m]andatory participation in PREP Act covered countermeasures" and contend "Defendants chang[ed] the voluntary nature" of a vaccination program into an "involuntary program," Compl. ¶¶ 377, 383, but they do not allege that they suffered any injury resulting from willful misconduct in the administration of a covered countermeasure such as a COVID vaccine (let alone that Shriners Children's administered the covered countermeasure). Absent a claim of "death or serious physical injury" proximately caused by "willful misconduct" in the administration of a covered countermeasure, Plaintiffs have failed to plead a claim under the PREP Act.

g. Plaintiffs' Breach of Contract Claim Fails.

Plaintiffs' breach of contract claim (Count 7) fails in two principal ways. First, the contract at issue—the CDC COVID Vaccination Program Provider

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Agreement, Compl. Ex. A, to which Plaintiffs are not parties—does not plausibly create a third-party beneficiary relationship with Plaintiffs, even construing all facts Plaintiffs allege as true. Second, the majority of the Shriners Children's Defendants were not parties to the contract, either, and thus cannot be responsible for any alleged breach. This too is fatal to Plaintiffs' claim.

(1) The Contract at Issue Does Not Make Plaintiffs Third-Party Beneficiaries.

A third-party beneficiary contract is created when "the parties [to the contract] intend that the promisor assume a direct obligation to the intended beneficiary at the time they enter into the contract." *Del Guzzi Constr. Co. v. Global Nw. Ltd.*, 105 Wn.2d 878, 886 (1986). Plaintiffs can assert a contract claim only under a third-party beneficiary theory, given that they are not parties to the contracts at issue. Under Washington law, "indirect reference to a third party does not make the third party a beneficiary of the [contract]." *Rajagopalan v. NoteWorld, LLC*, 718 F.3d 844, 847 (9th Cir. 2013). Rather, a court assesses "whether performance under the contract would necessarily and directly benefit that party." *Postlewait Const., Inc. v. Great Am. Ins. Cos.*, 106 Wn.2d 96, 99 (1986) (internal quotation marks omitted). "An incidental, indirect, or inconsequential benefit to a third party is insufficient to demonstrate an intent to create a contract directly obligating the promisor to perform a duty to a third party." *Kim v. Moffett*, 156 Wn. App. 689, 701 (2010) (citation omitted).

Here, Plaintiffs have not alleged, nor can they, that the contract to which they cite, the Provider Agreement—a contract expressly between Shriners

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Children's as an organization and the Centers for Disease Control about COVID vaccine administration, *see* Compl. Ex. A (ECF No. 1-1) at 2—contemplated Plaintiffs as third-party beneficiaries. Plaintiffs allege that the Provider Agreement promised them the "freedom to consider participation" in COVID vaccination "free from 'sanctions[.]'" Compl. ¶ 387. But the Provider Agreement itself does not say this, about Plaintiffs or anyone, and Plaintiffs have alleged no facts to suggest that Shriners Children's or the CDC intended that Shriners Children's assumed such a "direct obligation to [Plaintiffs] at the time they enter[ed] into the Contract." *Del Guzzi*, 105 Wn.2d at 886. Moreover, Shriners Children's performance under the Provider Agreement would not necessarily bear on Plaintiffs at all, as required by *Postlewait*, 106 Wn.2d at 99, because the purpose of the Provider Agreement focused on administration of the COVID vaccine, and Shriners Children's neither contracted to administer, nor in fact administered, the COVID vaccine to Plaintiffs. Any benefit from the Provider Agreement that Plaintiffs do or could plausibly plead is only incidental or indirect, which is insufficient to give Plaintiffs standing to sue as third-party beneficiaries. *Moffett*, 156 Wn. App. at 701.

Because Plaintiffs do not and cannot allege facts sufficient to show that the parties to the Provider Agreement intended to create a third-party beneficiary relationship with Plaintiffs, their breach of contract claim against all Shriners Children's Defendants must be dismissed.

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(2) The Individually Named Shriners Children's Defendants are Not Parties to the Contract.

It is axiomatic that a party cannot seek relief for breach of a contract from a person or entity not party to that contract. *See Grondal v. United States*, 513 F. Supp. 3d 1262, 1276 (E.D. Wash. 2021) ("Wapato Heritage cannot seek relief for the BIA's alleged breach of a contract where the BIA is not a party to the contract."). Regardless of the entity that signed on Shriners Children's behalf, the Provider Agreement existed between the organization and the CDC—not any of the individually named Shriners Children's Defendants. Because none of the individually named Shriners Children's Defendants were parties to the Provider Agreement, Plaintiffs' breach of contract claim against them must be dismissed.

h. Plaintiffs Do Not State Any Viable Claim in Alleging "Washington State Common Law Employment Torts."

Federal Rule of Civil Procedure 8(a)(2) requires "a short and plain statement of the claim showing that the pleader is entitled to relief." A complaint fails to state a claim when the plaintiff "does not identify what her claim is," *Turner v. BCForward*, 2022 WL 1078869, at *2 (W.D. Wash. Apr. 11, 2022), or otherwise places the Court "in the position of guessing what claims Plaintiff is attempting to bring or in supplying elements of claims not plead," *Ayers v. Sziebert*, 2008 WL 644880, at *2 (W.D. Wash. Mar. 6, 2008).

Here, like in *Turner* and *Ayers*, Plaintiffs do not identify what their claims actually are. Plaintiffs collectively assert "Washington State Common Law Employment Torts" as a claim (Count 8) without specifying what employment-

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related "civil wrongs" were allegedly committed. Tort, Black's Law Dictionary (11th ed. 2019). This is especially problematic given that Plaintiffs—numbering 12 former employees—have brought this nebulous non-claim against the eight Shriners Children's Defendants without specifying which of the plaintiffs or defendants are actually implicated, and for what conduct. *Cf. Ayers*, 2008 WL 644880, at *2 ("Plaintiff has named over twenty-five Defendants but does not identify what claims are being brought against which Defendants."). Plaintiffs' attempt to bring their "Employment Torts" claim impermissibly places the Court "in the position of guessing what claims Plaintiff[s] [are] attempting to bring [and] in supplying elements of claims not plead," *id.*, so the Court should dismiss the claim for its failure to satisfy Rule 8 requirements.

Even construing Plaintiffs' allegations liberally and affirmatively *searching* for a viable claim consistent with them—which is not Shriners Children's burden—still does not reveal any identifiable tort related to their employment with Shriners Children's. Termination for failure to comply with a company-wide vaccine policy does not give rise to a wrongful termination claim. *See Antunes*, 627 F. Supp. 3d at 567 (dismissing wrongful termination claim of employee fired for violating employer's COVID-19 vaccination policy); *Fallon v. Mercy Catholic Med. Ctr. of Se. Pa.*, 200 F. Supp. 3d 553 (E.D. Pa. 2016) (granting employer's motion to dismiss former employee's wrongful termination claim where employee refused to receive influenza vaccine as required by employer). Plaintiffs further do not allege that they were anything other than at-

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will employees, providing no other grounds for such a claim. *See White v. Relay Res.*, 2019 WL 5677541, at *2 (W.D. Wash. Oct. 31, 2019) (observing "the presumption that a Washington employment contract is at-will"). The Court should therefore dismiss Count 8 for its failure to comply with Rule 8, and because Plaintiffs' allegations are otherwise not consistent with any viable tort claim.

i. Plaintiffs' Outrage Claim Relies on Conclusory Allegations of Harm Related to Shriners Children's Lawful Vaccination Requirement.

The Court should dismiss Plaintiffs' outrage claim (Count 9) for multiple reasons, but chiefly because Shriners Children's institution of a lawful vaccination requirement cannot constitute "extreme and outrageous conduct" supporting an outrage claim. *See Cala v. Moorings Park Cmty. Health, Inc.*, 2022 WL 17405581, at *5 (M.D. Fla. Dec. 2, 2022) (plaintiff could not allege "outrageous conduct in support of a plausible IIED claim" in challenging termination for refusal to comply with vaccine requirement); *Cesare v. PACT MSO, LLC*, 2023 WL 5893429, at *5-6 (D. Conn. Sept. 11, 2023) ("no plausible IIED claim" when defendant's vaccination policies "were in line with those adopted by countless other employers across the country during the COVID-19 pandemic").

In addition to this fundamental flaw, Plaintiffs make only conclusory allegations regarding the supposed harm they suffered as a result of Shriners Children's vaccination policy. Compl. ¶¶ 310-311 (claiming Shriners Children's vaccination requirement "placed Plaintiffs in severe emotional distress, fearing

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for their lives and livelihoods"); *id.* ¶¶ 403-406 (claiming Shriners Children's engaged in a "scorched earth policy and inflicted with malicious intent severe emotional distress" on Plaintiffs).

The Court is not required to accept such "[c]onclusory allegations and unreasonable inferences" on a motion to dismiss. *Sanders v. Brown*, 504 F.3d 903, 910 (9th Cir. 2007). Plaintiffs allege no specific conduct by Shriners Children's that would establish the required elements to prove an outrage (intentional infliction of emotional distress) claim under Washington law: (1) extreme and outrageous conduct, (2) intentional or reckless infliction of emotional distress, and (3) actual result to plaintiff of severe emotional distress. *Kloepfel*, 149 Wn.2d at 195. An outrage claim "must be predicated on behavior ***so outrageous in character, and so extreme in degree, as to go beyond all possible bounds of decency, and to be regarded as atrocious, and utterly intolerable in a civilized community***." *Id.* at 196 (internal quotations omitted, emphasis in original).

Shriners Children's instituted a lawful vaccination requirement generally applicable to their employees after Governor Inslee issued Proclamation 21-14. As discussed, the U.S. Supreme Court has held that vaccination mandates generally (and for healthcare workers in particular) are constitutional and satisfy rational basis review. Even more, in an analogous case, the Washington Supreme Court held that Yakima health officials' threat to quarantine a tuberculosis patient pursuant to their authority under RCW ch. 70.28 was not outrageous as a matter

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of law, even after the patient died as a result of the treatment he received. *Reyes v. Yakima Health Dist.*, 191 Wn.2d 79, 91-92 (2018). Given the health officials' statutory responsibility for protecting the public from the spread of tuberculosis, "[t]he alleged conduct of the YHD employees was not wrongful, let alone outrageous." *Id.* at 92. The same is true here; Plaintiffs outrage claim should be dismissed.

j. No Private Right of Action Exists Under 21 U.S.C. § 360bbb-3 Supporting Plaintiffs' Claim.

The Court lastly should dismiss Plaintiffs' implied private right of action claim under 21 U.S.C. § 360bbb-3 (Count 10). Compl. ¶¶ 407-409. Plaintiffs contend that even if Shriners Children's were not engaged in state action, 21 U.S.C. § 360bbb-3 "contains an implied private right of action" such that Plaintiffs can assert a claim pursuant to the statute. Compl. ¶ 408. As discussed, courts nationally have consistently concluded that 21 U.S.C. § 360bbb-3 provides for ***no*** private right of action. *See supra* Section III.B.2.A.(2); *Smith*, 2023 WL 3569127, at *2; *Bridges*, 543 F. Supp. 3d at 527; *Kiss*, 2022 WL 17480936, at *8. There is no support for Plaintiffs' assertion that an implied private right of action exists under 21 U.S.C. § 360bbb-3.

C. The Court Should Dismiss Plaintiffs' Claims With Prejudice and Without Leave to Amend.

Plaintiffs' claims against Shriners Children's are fundamentally deficient and further amendment to their Complaint would be futile. Accordingly, the Court should dismiss their claims with prejudice and without leave to amend. *See Nafdel Petroleum Solutions Int'l LLC v. JP Morgan Chase Bank, NA*, 2019 WL

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13299133, at *2 (E.D. Wash. Aug. 27, 2019) (denying leave to amend when "no amendment could possibly cure the deficiencies of the Complaint"); *Book v. United States*, 2016 WL 6154932, at *3-4 (E.D. Wash. Oct. 21, 2016) (same). Further amendment would be futile with respect to Plaintiffs' six Section 1983 claims, as the Shriners Children's Defendants are not state actors, plus the law is settled that no private right of action exists under 21 U.S.C. § 360bbb-3 and courts routinely recognize that vaccination requirements are lawful. Plaintiffs additionally cannot establish any contractual relationship with Shriners Children's pursuant to the Provider Agreement, they fail to satisfy even minimal pleading requirements with their claim for supposed "Washington State Common Law Employment Torts" (and their allegations otherwise do not support a viable employment claim), and Shriners Children's institution of a lawful employee vaccination policy cannot provide a basis for an outrage claim. Dismissal with prejudice and without leave to amend is appropriate in these circumstances.

IV. CONCLUSION

For the foregoing reasons, the Court should dismiss Plaintiffs' claims against Shriners Children's in full and without leave to amend.

DATED this 15th day of December, 2023.

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CERTIFICATE OF SERVICE

I hereby certify that on the 15th day of December, 2023, I electronically filed the foregoing document with the Clerk of the Court using the CM/ECF system which will send notification of such filing to the following:

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